A · PATIENT DEMOGRAPHICS, VITALS & ANKLE / FOOT PRESENTATION PROFILE

INTAKE & VITALS

Patient Name _____ Age _____ Sex ☐ M ☐ F Side ☐ R ☐ L ☐ Bilat Date _____

Sport / Occupation / Footwear _____ Onset ☐ Acute trauma ☐ Gradual / Overuse NPRS Now 0 1 2 3 4 5 6 7 8 9 10

Vitals [E]: BP _____ HR _____ SpO_2 _____ Temperature _____ Blood glucose (if diabetic) _____

B · FRACTURE (OTTAWA RULES), ACHILLES RUPTURE, COMPARTMENT SYNDROME, RED FLAGS & MEDICATION / SAFETY STRIP

TRIAGE GATE

FRACTURE — OTTAWA ANKLE & FOOT RULES

Ottawa Calc

☐ **Ottawa ankle & foot rules (SR, 27 studies)** (Bachmann 2003 BMJ) [H]: **LR− 0.08 (0.03–0.18) ankle and 0.08 (0.03–0.20) mid-foot; children 0.07; < 1.4% probability of fracture at 15% prevalence; radiographs reduced by 30–40%** (bone tenderness at posterior edge or tip of distal 6 cm of lateral or medial malleolus, navicular, base of 5th MT, or inability to bear weight four steps)

☐ **Ottawa ankle rules — children (SR)** (Dowling 2009 Acad Emerg Med) [H]: **pooled sensitivity 98.5% (97.3–99.2); x-ray reduction 24.8%**

ACHILLES RUPTURE & COMPARTMENT SYNDROME

Maffulli 1998

☐ **Achilles rupture — clinical tests** ▸ & **triad** (Maffulli 1998 Am J Sports Med) [M]: **calf squeeze (Thompson) sensitivity 0.96, Matles 0.88; at least two tests positive in all patients with a tear → same-day referral**

☐ **Compartment syndrome (lower leg)** (Ulmer 2002 J Orthop Trauma) [L]: **positive predictive value of clinical findings 11% to 15%** (pain out of proportion / passive stretch pain → emergent surgical referral)

RED FLAGS IN WORDS & MEDICATION / SAFETY STRIP

Wise 2012

☐ **Same-day red flags** [E]: **DVT** (calf swelling, warmth, tenderness); **septic joint** (hot, swollen joint, fever, inability to bear weight); **Lisfranc injury** ▸ (plantar midfoot ecchymosis, TMT instability); **neuropathic / Charcot foot** (red, hot, swollen foot in neuropathy); **infection / spreading cellulitis**; **ischaemic foot** (pulseless, pale, cold, rest pain)

☐ **Fluoroquinolones & Achilles rupture** (van der Linden 2003 Arch Intern Med · Wise 2012 Am J Med) [M]: **adjusted OR 4.3 current, 2.4 recent exposure; OR 6.4 (60–79 years) and 20.4 (≥ 80 years); 2 % to 6 % of ruptures after 60 attributable to quinolones · tendonitis OR 4.3, rupture OR 2.0; stronger with oral glucocorticoids (OR 9.1) and age > 60 (OR 8.3)**

☐ **Anticoagulants / coagulopathy before needling** (Kearns 2019 J Man Manip Ther) [E]: **caution — possible increased incidence or severity of haematoma and bruising**

☐ **Electrophysical-agent contraindications** (Houghton 2010 Physiother Can) (Guideline): **implanted electronic devices, pregnancy, malignancy, recently irradiated tissue, active DVT, bleeding, impaired sensation**

TRIAGE DISPOSITION: ☐ **Cleared — proceed** ☐ Radiographs required (Ottawa positive)
☐ **Same-day referral (Achilles rupture / compartment / DVT / septic / Lisfranc / Charcot / ischaemic)**

C · CLINICAL PATHWAY ROUTER & ANKLE / FOOT SUBGROUP NAVIGATION

RAPID

CLINICAL PATHWAY ROUTER — Tick route → jump to matching pages

☐ A · SPRAIN / CAI: 1 → 2 → 4 → 7 → 8 ☐ B · TENDON / HEEL: 1 → 2 → 4 → 6 → 8 ☐ C · FRACTURE / POST-OP: 1 → 3 → 5 → 8 → 9

D · SUBJECTIVE HISTORY, SYMPTOM BEHAVIOUR & ANKLE / FOOT BODY CHART

SUBJECTIVE

AGGRAVATING, EASING & IRRITABILITY PROFILE

Behaviour [E]

Aggravating _____ Easing _____

Irritability: ☐ Low ☐ Moderate ☐ High (first-step / night / rest pain)

MECHANISM & RISK FACTORS

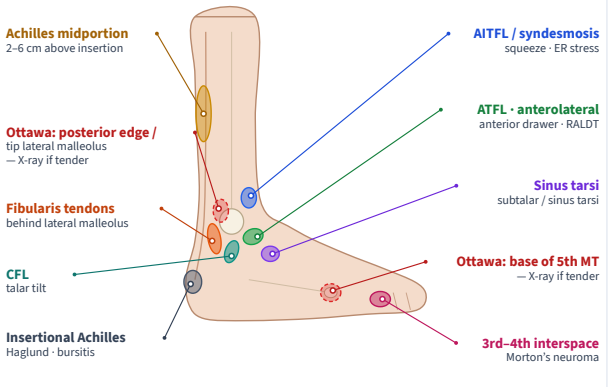
History [E]

Mechanism: ☐ Inversion ☐ DF / ER / eversion ☐ Push-off snap ☐ Load spike

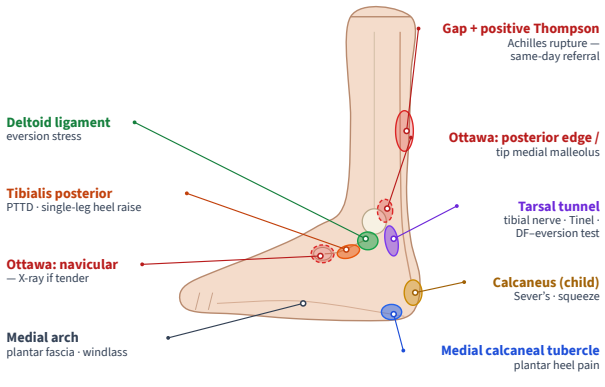
Factors: ☐ Prior sprains / giving way ☐ Fluoroquinolone / steroid ☐ Anticoagulant / diabetes

BODY CHART — WHERE IS THE PAIN?

Clinical signposts [E]



LATERAL · RIGHT ANKLE



MEDIAL · RIGHT ANKLE

Mark on the chart: shade = pain · //// = pins & needles · ×× = numbness · → = radiation · write NPRS beside each area

Ottawa bone tenderness or unable to bear weight four steps → radiographs. **Gap + positive Thompson** ▸ → Achilles rupture (same-day referral).

SUBJECTIVE HISTORY · MECHANISM · PATIENT GOALS (PSFS)

Clinician _____ Date _____

Underline = Atlas card · ▸ = video · Evidence: [H] high [M] moderate [L] low [E] expert · Confidential · v2026.10

A · ANKLE & FOOT DIFFERENTIAL DIAGNOSIS MATRIX (LIGAMENT, TENDON, BONE, JOINT & NERVE)

DIFFERENTIAL

CONDITION	HALLMARK HISTORY & PAIN LOCATION	KEY PHYSICAL FINDINGS & PROVOCATION	EVIDENCE & ATLAS
Lateral ankle sprain	Inversion injury; anterolateral swelling, ecchymosis and tenderness	Delayed day-5 exam, anterior drawer ▸, RALDT, talar tilt ▸; deltoid / sinus tarsi check	M Sn 96%, Sp 84% (van Dijk 1996)
Syndesmosis sprain	Dorsiflexion / external-rotation or eversion trauma; pain above ankle mortise	Squeeze test ▸ (LR+ 2.20, LR− 0.68), DF–ER stress ▸, fibular translation ▸, inability to hop	M Sman 2015 · Baltes 2022
Chronic ankle instability (CAI)	Recurrent giving way, perceived instability and persistent disability after sprain	CAIT ≤ 25 (Sn 96.6%, Sp 86.8%); 2-wk drop landing / vertical jump; 6-mo SEBT + FAAM-ADL	M Doherty 2016 · Wright 2014
Midportion vs insertional Achilles tendinopathy	Gradual loading pain: midportion 2–6 cm above insertion vs posterior calcaneal tuberosity	Palpation (Sn 84%, Sp 73%), pain 2–6 cm (78% / 77%), arc sign ▸, Royal London ▸ (C); insertional bony prominence	M Hutchison 2013 · Martin 2018 (C)
Achilles tendon rupture ▸	Sudden pop / push-off snap in calf; immediate weakness and limp	Thompson ▸ Sn 0.96, Matles 0.88, palpable gap; ≥ 2 positive in all tears → same-day referral	M Maffulli 1998
Plantar heel pain	Plantar medial heel pain worst on first steps in the morning or after prolonged sitting	Medial calcaneal tubercle palpation; weight-bearing windlass ▸ positive in 31.8% (low sensitivity)	L De Garceau 2003
Morton's neuroma	Forefoot burning or shooting pain / paraesthesia in the 3rd–4th interspace	Thumb–index finger squeeze Sn 96%; Mulder's click ▸ Sn 61%	L Mahadevan 2015
Tarsal tunnel syndrome	Medial ankle burning, tingling or numbness radiating to the plantar foot	Dorsiflexion–eversion tibial nerve provocation & Tinel behind medial malleolus; Slump test ▸ for radicular source	E Clinical differentiation
Tibialis posterior dysfunction	Posteromedial ankle pain and swelling with progressive acquired flatfoot	Tendon tenderness behind medial malleolus; weak single-leg heel-rise / absent heel inversion	E Clinical differentiation
Sever's apophysitis (child)	Activity-related posterior heel pain in a growing child or adolescent	Medial–lateral calcaneal squeeze reproduces familiar posterior heel pain	E Clinical differentiation
Navicular / 5th MT stress fracture ▸	Insidious dorsal midfoot or lateral foot pain worsening with running / impact	Focal bony tenderness at dorsal navicular "N-spot" or 5th MT base / proximal shaft	E Clinical differentiation
Osteochondral lesion (OCL)	Deep ankle ache, catching, locking or persistent effusion after sprain or trauma	Joint-line tenderness, effusion; bone marrow stimulation RTS 83.3%, pre-injury level 52.7%	L Luca 2025
Lisfranc injury ▸	Midfoot twist or axial load on a plantarflexed foot; midfoot swelling and inability to bear weight	Plantar arch ecchymosis; TMT palpation and passive pronation–abduction provocation	E Clinical differentiation
Anterolateral impingement	Persistent anterolateral ankle pain at end-range dorsiflexion after inversion sprain	Anterolateral impingement sign (thumb pressure PF → DF): Sn 94.8%, Sp 88%	L Molloy 2003

B · EPIDEMIOLOGICAL INCIDENCE, CAI PREDICTION & DIAGNOSTIC ACCURACY EVIDENCE

PROGNOSIS & ACCURACY

SPRAIN INCIDENCE, CAI PREDICTION & SYNDESMOSIS

Doherty 2014

- ☐ **Ankle sprain incidence (MA, 181 studies)** (Doherty 2014 Sports Med) H: **females 13.6 vs males 6.94 per 1,000 exposures; highest in indoor / court sports**
- ☐ **Predicting chronic ankle instability (cohort)** (Doherty 2016 Am J Sports Med) M: **inability to complete drop landing and drop vertical jump at 2 weeks: 67.6% correctly classified (sensitivity 83%, specificity 55%); 6-month SEBT posterior reach + FAAM-ADL: 84.8%**
- ☐ **Syndesmosis — test combination** (Sman 2015 Br J Sports Med) M: **inability to hop, syndesmosis tenderness and dorsiflexion–external rotation stress (sensitive) + pain out of proportion and squeeze test (specific)**
- ☐ **Syndesmosis — acute evaluation (MRI reference)** (Baltes 2022 Knee Surg Sports Traumatol Arthrosc) M: **squeeze test LR+ 2.20, LR− 0.68; eversion mechanism LR+ 3.47; overall clinical suspicion sensitivity 73%, NPV 89%**

IMPINGEMENT, ACHILLES, WINDLASS & MORTON'S NEUROMA

Hutchison 2013

- ☐ **Anterolateral impingement sign** (Molloy 2003 J Bone Joint Surg Br) L: **sensitivity 94.8%, specificity 88%**
- ☐ **Midportion Achilles tendinopathy — best tests** (Hutchison 2013 Foot Ankle Surg) M: **pain on palpation sensitivity 84%, specificity 73%; pain 2–6 cm above insertion 78% / 77%**
- ☐ **Windlass test — plantar fasciitis** ▸ (De Garceau 2003 Foot Ankle Int) L: **positive weight-bearing in 31.8% of patients (low sensitivity)**
- ☐ **Morton's neuroma (ultrasound reference)** (Mahadevan 2015 J Foot Ankle Surg) L: **thumb–index finger squeeze sensitivity 96%; Mulder's click 61%**

Primary Working Diagnosis _____ Secondary / Contributing Factors _____ Prognostic / CAI Risk Notes _____

DIFFERENTIAL REASONING · PROGNOSTIC COUNSELLING · RISK-FACTOR NOTES

A · OBJECTIVE MEASURES (WBLT, FPI-6, SINGLE-LEG HEEL-RISE & JOSPT CPG IMPAIRMENT MEASURES)

IMPAIRMENT MEASURES

WEIGHT-BEARING LUNGE, FOOT POSTURE & HEEL-RISE

Bennell 1998

- ☐ **Weight-bearing lunge test** ▸ (Bennell 1998 Aust J Physiother) [M]: **intra-rater ICC 0.97–0.98; inter-rater ICC 0.97 (angle) and 0.99 (distance)**
- ☐ **Foot Posture Index — norms** (Redmond 2008 J Foot Ankle Res · Rokkedal-Lausch 2013 Gait Posture) [M]: **healthy adults: slightly pronated, mean score +4; normal left–right difference −2 to +2**
- ☐ **Single-leg heel-rise test (10° incline, to fatigue)** (Hébert-Losier 2017 Physiotherapy) [M]: **ICC 1.0; men median 24 vs women 21 repetitions; women over 60 outperformed men**

JOSPT LAS CPG IMPAIRMENT MEASURES (MARTIN 2021)

JOSPT 2021

- ☐ **Impairment measures** (Martin 2021 J Orthop Sports Phys Ther) (Guideline): **dorsiflexion "using the weight-bearing lunge test", "static single-limb balance on a firm surface with eyes closed" and the Star Excursion Balance Test (A); single-limb hopping under timed conditions (B)**

WBLT (cm / °) Inv / Uninv _____ FPI-6 Inv / Uninv _____

Heel-Rise Reps Inv / Uninv _____ SLS Eyes Closed / SEBT / Hop _____

B · ANKLE & FOOT RANGE OF MOTION WRITE-IN TABLE (INVOLVED VS UNINVOLVED)

GONIOMETRY

MOVEMENT (ROM) [E]	INVOLVED AROM	INVOLVED PROM	UNINVOLVED	END-FEEL / SYMPTOM RESPONSE
Talocrural Dorsiflexion (knee ext / flex)				☐ Firm ☐ Bony ☐ Pain
Talocrural Plantarflexion				☐ Firm ☐ Caps ☐ Pain
Subtalar / Rearfoot Inversion				☐ Firm ☐ Lax ☐ Pain
Subtalar / Rearfoot Eversion				☐ Firm ☐ Bony ☐ Pain
1st MTP Dorsiflexion / Plantarflexion				☐ Firm ☐ Bony ☐ Pain

☐ **Peripheral joint ROM reference database** (Soucie 2011 Haemophilia) [M]: **significant age- and sex-related variation — compare with the other side and the patient's own baseline**

C · HAND-HELD DYNAMOMETER (HHD) STRENGTH, TORQUE & LIMB SYMMETRY INDEX (LSI) WRITE-IN TABLE

HHD & TORQUE

ISOMETRIC HHD WRITE-IN

Torque Calc LSI Calc

MOVEMENT	LEVER ARM (CM)	FORCE (N) INV / UNINV	TORQUE (N·M) = N × M	LSI % = INVOLVED ÷ UNINVOLVED × 100
Plantarflexion				
Dorsiflexion				
Inversion				
Eversion				
Hip abduction				

☐ **HHD reliability rule** (Martins 2017 J Athl Train · Batista 2024 Phys Ther Sport) [M]: **use the same tester, same position, same strap / lever-arm point and belt / fixed set-up when possible; compare with the other side and do not interchange devices**

OBJECTIVE EXAMINATION · WBLT / FPI-6 / HEEL-RISE · GONIOMETRY · HHD TORQUE & LSI NOTES

A · ANKLE & FOOT SPECIAL TESTS (LATERAL LIGAMENT, SYNDESMOSIS, ACHILLES, PLANTAR FASCIA & FOREFOOT / MIDFOOT)

SPECIAL TESTS

REGION & TEST	TECHNIQUE & POSITIVE CRITERION	DIAGNOSTIC PERFORMANCE & CPG GRADE	RESULT
Delayed day-5 exam, anterior drawer ▸, RALDT & talar tilt ▸	ATFL palpation + haematoma + anterior drawer at day 5; reverse anterolateral drawer; inversion talar tilt (CFL)	Day-5 exam **Sn 96%, Sp 84%; kappa 0.5–1.0** (van Dijk 1996) M · LAS CPG (B) (Martin 2021)	☐ – ☐ +
Squeeze test ▸, DF–ER stress ▸ & fibular translation ▸	Compress tibia/fibula at mid-calf; dorsiflexion + external rotation of foot; A–P fibular shuck	Squeeze **LR+ 2.20, LR− 0.68**; eversion mechanism **LR+ 3.47**; clinical suspicion **Sn 73%, NPV 89%** (Baltes 2022) M · Sman 2015 M	☐ – ☐ +
Arc sign ▸ & **Royal London test** ▸	Intratendinous swelling moves with DF/PF; tenderness reduces in maximum dorsiflexion	Palpation **Sn 84%, Sp 73%**; pain 2–6 cm **78% / 77%** (Hutchison 2013) M · Achilles CPG (C) (Martin 2018)	☐ – ☐ +
Windlass test ▸	Passive 1st MTP extension in weight-bearing reproduces plantar heel pain	Positive weight-bearing in **31.8%** of patients (low sensitivity) (De Garceau 2003) L	☐ – ☐ +
Mulder's click ▸ & interspace squeeze	Transverse forefoot compression + thumb–index web-space squeeze (3rd–4th interspace)	Thumb–index squeeze **Sn 96%**; Mulder's click **Sn 61%** (Mahadevan 2015) L	☐ – ☐ +
Navicular drop ▸ & **Lisfranc test** ▸	Seated subtalar-neutral vs standing navicular height; midfoot passive pronation–abduction / TMT palpation	Medial arch excursion & tarsometatarsal stability E	☐ – ☐ +

☐ **JOSPT LAS CPG — diagnosis** (Martin 2021 J Orthop Sports Phys Ther) (Guideline): **"Clinicians should use special tests, including the reverse anterolateral drawer test and anterolateral talar palpation in addition to the traditional anterior drawer test" (B)** · CAI: use a discriminative instrument **"such as the Cumberland Ankle Instability Tool or the Identification of Functional Ankle Instability" plus functional performance tests (B)**

☐ **JOSPT Achilles CPG — diagnosis** (Martin 2018 J Orthop Sports Phys Ther) (Guideline): in addition to the arc sign and Royal London Hospital test: **"pain located 2 to 6 cm proximal to the Achilles tendon insertion that began gradually and pain with palpation of the midportion of the tendon" (C)**

B · VALIDATED ANKLE & FOOT PROM DASHBOARD (FAAM, CAIT, VISA-A, LEFS, ATRS, NPRS, PSFS) & CPG QUOTES

PROMS

OUTCOME SCALE	SCORING RULE & SUBSCALES	RELIABILITY, MDC, MCID & CUT-OFFS	INITIAL	FOLLOW-UP
FAAM Calc	21-item ADL (0–84) and 8-item Sports (0–32) subscales → 0–100%	Test–retest **0.89 / 0.87**; **MDC ±5.7 (ADL) and ±12.3 (Sports)** (Martin 2005) M		
CAIT Calc	9 items, scored per ankle (0–30; lower score = greater instability)	Original threshold **27.5 (Sn 82.9%, Sp 74.7%), ICC .96** (Hiller 2006); recalibrated **≤25 for CAI: Sn 96.6%, Sp 86.8%** (Wright 2014) M		
VISA-A Calc	8 items (stiffness, pain, function, sport; 0–100)	Test–retest **r = 0.93; controls exceeded 96** (Robinson 2001) M		
LEFS Calc	20 lower-extremity activities (0–4 each; total 0–80)	Test–retest **R = .94; MDC 9 and MCID 9 points** (Binkley 1999) M		
ATRS	Patient-reported outcome after Achilles tendon rupture	Validated patient-reported score after Achilles rupture (Nilsson-Helander 2007) M		
NPRS Calc · **PSFS** Calc	NPRS 0–10 (now / best / worst) · PSFS mean of 3–5 patient goals (0–10)	Pain intensity & patient-specific goal tracking E		

☐ **JOSPT LAS CPG — outcome measures** (Martin 2021 J Orthop Sports Phys Ther) (Guideline): **"Clinicians should use validated patient-reported outcome measures, such as the … Foot and Ankle Ability Measure, and the Lower Extremity Functional Scale" (A)**

☐ **JOSPT Achilles CPG — outcome measures** (Martin 2018 J Orthop Sports Phys Ther) (Guideline): **"Clinicians should use the Victorian Institute of Sport Assessment-Achilles (VISA-A)" and either the FAAM or the LEFS (A); hop and heel-raise endurance tests (B)**

SPECIAL TESTS SYNTHESIS · BASELINE PROM SCORES · FUNCTIONAL GOAL SETTING

Clinician _____ Date _____

Underline = Atlas card · ▸ = video · Evidence: H high M moderate L low E expert · Confidential · v2026.10

A · POST-SURGICAL EVIDENCE: ACHILLES RUPTURE, LATERAL LIGAMENT REPAIR (BROSTRÖM) & TALAR OSTEOCHONDRAL LESIONS

POST-SURGICAL

POST-OP RESTRICTIONS ARE SURGEON-DIRECTED [E]

ACHILLES RUPTURE — OPERATIVE VS NON-OPERATIVE

Ochen 2019

- ☐ **Operative vs non-operative (MA, 29 studies)** (Ochen 2019 BMJ) [H]: **re-rupture 2.3% vs 3.9% (RR 0.43); complications 4.9% vs 1.6% (RR 2.76); with accelerated functional rehabilitation no significant re-rupture difference (RR 0.60)**
- ☐ **Accelerated non-operative protocol (RCT)** (Willits 2010 J Bone Joint Surg Am) [H]: **outcomes clinically similar to operative treatment, avoiding surgical complications**
- ☐ **RCT meta-analysis (early weight-bearing studies)** (Xu 2025 Ann Med) [M]: **fewer re-ruptures with minimally invasive (RR 0.28) and open repair (RR 0.17); open repair complications RR 3.03; minimally invasive sural nerve injury RR 5.86**

LATERAL LIGAMENT REPAIR & OSTEOCHONDRAL LESION

Cho 2025

- ☐ **Broström + suture tape (case series)** (Coetzee 2018 Foot Ankle Int) [L]: **mean return to sport 84.1 days; single-leg hop LSI normal / near normal in 86.4%; FAAM Sports ≥ 90 in 79.0%**
- ☐ **Modified Broström + suture tape (≥ 3 years, n = 94)** (Cho 2025 J Clin Med) [L]: **recurrent instability 2.1%; FAAM 90.5 at final follow-up**
- ☐ **Talar osteochondral lesions — bone marrow stimulation (MA)** (Luca 2025 Clin Sports Med) [L]: **return to sport 83.3%; to pre-injury level 52.7%**

B · CRITERIA TO PROGRESS CHECKLIST & WOUND / NEUROVASCULAR / BOOT-COMPLAINT RE-CHECK

PROGRESSION GATE

CRITERIA TO PROGRESS BETWEEN REHABILITATION STAGES

Expert [E]

- ☐ **Surgeon clearance for this stage** documented in operative / clinic notes [E]
- ☐ **Wound healed** (incisions dry, sutures / staples cleared, no erythema or drainage) [E]
- ☐ **Effusion / swelling settled** compared with prior visit and unaffected ankle [E]
- ☐ **Pain within the green / amber zone (p8)** during and the morning after loading [E]
- ☐ **ROM vs other side** progressing within surgeon-allowed range [E]
- ☐ **Strength LSI recorded on p3** (HHD torque & single-leg heel-rise) [E]
- ☐ **Movement quality** symmetrical during gait, squat, heel-rise and landing tasks [E]
- ☐ **Patient confidence** and psychological readiness for the next functional demand [E]

Wound / Neurovascular / Boot Re-Check [E]: ☐ Wound erythema / dehiscence / discharge ☐ Sural / tibial / fibular nerve paraesthesia ☐ Calf swelling / tenderness (DVT screen) ☐ Boot / cast pressure sore or rubbing

C · OPERATIVE & SURGEON-DIRECTED RESTRICTIONS LOG (BLANK WRITE-IN)

OPERATIVE LOG

Procedure / Graft / Fixation _____ **Surgery Date** _____ **Surgeon / Hospital** _____

Surgeon's Restrictions as Written (Weight-Bearing, Boot / Wedges / Orthosis, ROM Limits) _____

Wound & Sural / Distal Neurovascular Status _____ **Next Surgical Review Date** _____

POST-OPERATIVE COMMUNICATION · SURGEON ORDERS · PROGRESSION GATE NOTES

A · EXTRACORPOREAL SHOCKWAVE THERAPY (ESWT), CORTICOSTEROID INJECTIONS & JOSPT CPG QUOTES

ESWT, INJECTION & CPG

SHOCKWAVE & CORTICOSTEROID INJECTION EVIDENCE

Aqil 2013

- ☐ **Shockwave for chronic plantar fasciitis (MA)** (Aqil 2013 Clin Orthop Relat Res) [M]: **VAS reduction SMD 0.60 (0.34–0.85); 60% improvement on first steps RR 1.30**
- ☐ **Focused shockwave for plantar fasciitis (MA)** (Sun 2017 Medicine (Baltimore)) [M]: **improvement rate OR 2.58 (1.97–3.39) vs placebo**
- ☐ **Achilles tendinopathy — eccentric vs shockwave vs wait-and-see (RCT)** (Rompe 2007 Am J Sports Med) [M]: **"completely recovered / much improved" 60% vs 52% vs 24%; eccentric and low-energy shockwave comparable**
- ☐ **Corticosteroid injection — plantar heel pain (MA)** (Whittaker 2019 BMC Musculoskelet Disord) [M]: **similar to placebo injection short and medium term; less effective than dry needling in the longer term (SMD 1.45)**

JOSPT CPG QUOTES — LAS 2021, ACHILLES 2018 & HEEL PAIN 2023

Koc 2023

- ☐ **LAS CPG — physical agents** (Martin 2021 JOSPT) (Guideline): **ice with exercise (C); pulsed shortwave diathermy (C); low-level laser for initial pain (C); electrotherapy conflicting (D); "Clinicians should not use ultrasound for the management of acute ankle sprains" (A)**
- ☐ **Achilles CPG — other interventions** (Martin 2018 JOSPT) (Guideline): **stretching (C); iontophoresis with dexamethasone for acute presentations (B); "should not use night splints" (C); "should not use therapeutic elastic tape" (F); heel lifts, orthoses, low-level laser — no recommendation, contradictory evidence (D)**
- ☐ **Heel pain CPG — orthoses, night splints, physical agents & scope** (Koc 2023 JOSPT) (Guideline): **"should not use orthoses … as an isolated treatment for short-term pain relief" (B); orthoses combined with other treatment (C); "a 1- to 3-month program of night splints" for first-step morning pain (A) · "should not use ultrasound to enhance the benefits of stretching" (A); low-level laser as part of rehabilitation for short-term pain (B); phonophoresis with ketoprofen (C); electrotherapy second line — iontophoresis or premodulated interferential (D) · shockwave, corticosteroid and PRP injection were outside the CPG scope (scoping summary only)**

B · BLOOD FLOW RESTRICTION (BFR) POSITION STAND & ELECTROPHYSICAL-AGENT CONTRAINDICATION REMINDER

BFR & SAFETY

BLOOD FLOW RESTRICTION (BFR)

Patterson 2019

- ☐ **Position stand — pressure, load & cuff width** (Patterson 2019 Front Physiol) (Guideline): **40 to 80 % of arterial occlusion pressure (AOP), set from a measured AOP; 20–40 % 1RM; 75 repetitions as 30-15-15-15; rest 30–60 s; 2–3 sessions per week; a wide variety of cuff widths can be used if pressure is set from AOP**
- ☐ **Sports medicine BFR (review)** (Lorenz 2021 J Athl Train) [L]: **resistance exercise typically 20 % to 40 % of 1-RM**
- ☐ **Cuff placement** [E]: **Cuff on the PROXIMAL THIGH only**

ELECTROPHYSICAL-AGENT CONTRAINDICATIONS (HOUGHTON 2010)

Houghton 2010

- ☐ **Contraindication reminder** (Houghton 2010 Physiother Can) (Guideline): **agent-by-agent table (contraindicated / over the site / precaution / safe): implanted electronic devices, pregnancy, malignancy, recently irradiated tissue, active DVT, bleeding, impaired sensation**

C · EPA, SHOCKWAVE, BFR, ORTHOSIS & INJECTION DELIVERY LOG (BLANK WRITE-IN)

PARAMETER LOG

DATE	MODALITY / ORTHOSIS / BFR / INJECTION	SITE / AOP % / LOAD / WEAR SCHEDULE	PRE / POST NPRS	PATIENT RESPONSE

EPA / ESWT / BFR PARAMETERS · ORTHOSIS & NIGHT-SPLINT SCHEDULE · INJECTION NOTES

Clinician _____ Date _____

Underline = Atlas card · ▸ = video · Evidence: [H] high [M] moderate [L] low [E] expert · Confidential · v2026.10

A · JOINT MOBILISATION, STRETCHING, TAPING & DRY NEEDLING EVIDENCE

MANUAL & NEEDLING

MANUAL THERAPY, STRETCHING & TAPING

Weerasekara 2018

- ☐ **Joint mobilisation for ankle sprain (MA)** (Weerasekara 2018 Arch Phys Med Rehabil) [M]: **immediate benefit for posteromedial dynamic balance; short-term improvement in weight-bearing dorsiflexion (P = .003)**
- ☐ **LAS CPG — manual therapy (acute & CAI)** (Martin 2021 JOSPT) (Guideline): **acute sprain: manual therapy within pain-free movement alongside exercise (A) · CAI: manual therapy incl. mobilization with movement for dorsiflexion and balance (A); external support not as a stand-alone (B)**
- ☐ **Heel pain CPG — manual therapy, stretching & taping** (Koc 2023 JOSPT) (Guideline): **manual therapy to lower-extremity joints and soft tissue (A); plantar-fascia-specific and gastrocnemius / soleus stretching (A); rigid or elastic taping with other treatment for short-term pain and function (A)**

DRY NEEDLING EVIDENCE & CPG GRADES

Cotchett 2014

- ☐ **Dry needling for plantar heel pain (RCT, 1 per week × 6)** (Cotchett 2014 Phys Ther) [M]: **first-step pain −14.4 mm vs sham (below minimal important difference); NNT 4; minor transient adverse events 32%**
- ☐ **LAS CPG — fibularis needling for CAI** (Martin 2021 JOSPT) (Guideline): **dry needling of the fibularis group with proprioceptive training (C)**
- ☐ **Heel pain CPG — dry needling** (Koc 2023 JOSPT) (Guideline): **"dry needling to MTrP in the gastrocnemius, soleus, and plantar muscles" for short- and long-term pain (B)**

B · DRY NEEDLING SAFETY BOX (ADVERSE EVENTS, SERIOUS RISKS & ANTICOAGULANTS)

NEEDLING SAFETY

ADVERSE EVENT RATES (PROSPECTIVE STUDIES)

Brady 2014

- ☐ **Adverse events — 7,629 treatments** (Brady 2014 J Man Manip Ther) [M]: **mild AEs 19.18 %; upper risk rate for significant AEs ≤ 0.04 %; bruising 7.55 %, bleeding 4.65 %**
- ☐ **Adverse events — 20,464 treatments** (Boyce 2020 Int J Sports Phys Ther) [M]: **minor AE 36.7 %; major AE < .1 %; most common major: prolonged symptom aggravation .03 %, fainting .02 %, forgotten needles .01 %**

SERIOUS RISKS & ANTICOAGULANT CAUTION

Kearns 2019

- ☐ **Known serious risks** (Halle 2016 Int J Sports Phys Ther) [E]: **pneumothorax, cardiac tamponade, haematoma, infection, CNS injury; filament needles 32–36 gauge (.25–.20 mm)**
- ☐ **Anticoagulants / coagulopathy** (Kearns 2019 J Man Manip Ther) [E]: **caution — possible increased incidence or severity of haematoma and bruising**

C · INFORMED CONSENT & MANUAL THERAPY / DRY NEEDLING TREATMENT LOG

CONSENT & LOG

CONSENT OBTAINED FOR: ☐ Talocrural / subtalar / midfoot mobilisation (MWM) ☐ Soft-tissue / plantar fascia / calf stretching & taping ☐ Dry needling (risks, bruising / bleeding & anticoagulant check explained)

Patient / Guardian Signature _____ **Clinician** _____ **Date** _____

DATE	TECHNIQUE (JOINT MOB / MWM / TAPING / NEEDLING)	TARGET STRUCTURE & DOSAGE	PRE / POST WBLT OR NPRS	ADVERSE EVENTS

MANUAL THERAPY RESPONSE · MWM DIRECTION · DRY NEEDLING SITE & POST-NEEDLING CARE NOTES

A · TENDON & FASCIA LOADING, LATERAL ANKLE SPRAIN EXERCISE / PREVENTION & PAIN-MONITORING TRAFFIC LIGHT

EXERCISE & PAIN MODEL

ACHILLES & PLANTAR HEEL LOADING + CPG QUOTES

Beyer 2015

- ☐ **Achilles tendinosis — heavy-load eccentric (12 weeks)** (Alfredson 1998 Am J Sports Med) [L]: **all 15 patients back to preinjury running**
- ☐ **Achilles tendinopathy — HSR vs eccentric (RCT)** (Beyer 2015 Am J Sports Med) [H]: **equally good, lasting results; HSR greater satisfaction at 12 weeks**
- ☐ **Plantar fasciitis — high-load strength (RCT)** (Rathleff 2015 Scand J Med Sci Sports) [M]: **FFI 29 points lower than stretching at 3 months; no difference at 1, 6, 12 months**
- ☐ **Achilles CPG — exercise & activity modification** (Martin 2018 JOSPT) (Guideline): **"mechanical loading, which can be either in the form of eccentric exercise, or a heavy-load, slow-speed (concentric/eccentric) exercise program" (A); "exercise at least twice weekly within their pain tolerance" (F) · complete rest is not indicated — continue recreational activity within pain tolerance (B)**
- ☐ **Heel pain CPG — resistance training & lean body mass** (Koc 2023 JOSPT) (Guideline): **resistance training for foot and ankle musculature (B); education on lean body mass (E)**

LAS CPG EXERCISE / PREVENTION & PAIN TRAFFIC LIGHT

JOSPT 2021

- ☐ **LAS CPG — prevention & exercise** (Martin 2021 JOSPT) (Guideline): **prophylactic bracing to reduce first-time sprain (A); after a first sprain: bracing + proprioceptive and balance-focused exercise (A) · acute: "use external supports, such as braces or taping, and … progressively bear weight" (A); structured therapeutic exercise (A) · CAI: proprioceptive / neuromuscular exercise (A)**
- ☐ **Pain-monitoring model** (Sprague 2021 Pilot Feasibility Stud · Cavaggion 2024 Open Access J Sports Med · Silbernagel 2007 Am J Sports Med): **pain should not exceed 5/10 on the NPRS during or immediately after activity and should return to pre-activity levels by the following morning (E); low (0–2), acceptable (2–5) and high (5–10) risk zones were based on clinical experience (E); continued tendon loading with a pain-monitoring model showed no negative effects (M)**

OK / Continue [E]: _____

Modify (load / volume / surface) [E]: _____

Avoid (for now) [E]: _____

B · RETURN TO SPORT — PAASS CONSENSUS CHECKLIST (SMITH 2021) & HOP / HEEL-RISE LSI WRITE-IN

RETURN TO SPORT

☐ **PAASS consensus domains** (Smith 2021 Br J Sports Med) (Guideline): tick each domain cleared before unrestricted return to sport:

☐ **1. Pain** (during & next-morning response) ☐ **2. Ankle impairments** (ROM, strength, endurance) ☐ **3. Athlete perception** (confidence, CAIT / FAAM) ☐ **4. Sensorimotor control** (proprioception, dynamic balance) ☐ **5. Sport / functional performance** (hopping, cutting, landing)

Single-Leg Hop Inv / Uninv & LSI % _____ Single-Leg Heel-Rise Inv / Uninv & LSI % _____ RTS Clearance ☐ Train ☐ Full Match

C · 12-WEEK ANKLE & FOOT PROGRESSIVE LOADING LOG (BLANK WRITE-IN)

12-WEEK LOG

EXERCISE / DRILL	WK 1	WK 2	WK 3	WK 4	WK 5	WK 6	WK 7	WK 8	WK 9	WK 10	WK 11	WK 12
Gastroc / Soleus Heel-Rise (HSR / Ecc)												
Plantar Fascia High-Load (Rathleff)												
Inversion / Eversion / Intrinsic Strength												
Single-Limb Balance / SEBT / Perturbation												
Plyometric / Hopping / Deceleration												

HOME EXERCISE PRESCRIPTION · PAIN-MONITORING COACHING · RETURN-TO-SPORT NOTES

Clinician _____ Date _____

Longitudinal Session-by-Session Tracking Flowsheet & Session 6 Checkpoint

Patient _____

#	DATE	NPRS	WBLT	ROM / FPI	HHD TORQUE / LSI	EXERCISE & LOADING DELIVERED	MANUAL / NEEDLING / EPA	PAIN ZONE	PROM	PATIENT SIGNATURE
1						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
2						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
3						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
4						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
5						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
6						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
7						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
8						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
9						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
10						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
11						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		
12						HSR/Ecc | Load | Bal | Hop	MWM | Tape | DN | EPA	0–2 | 2–5 | 5–10		

SESSION 6 CHECKPOINT & DISCHARGE MILESTONES (MDC & MCID BENCHMARKS)

CHECKPOINT & DISCHARGE

☐ **Session 6 checkpoint** [E]: re-score the main PROM against its MDC / MCID (**FAAM MDC ±5.7 ADL and ±12.3 Sports**, Martin 2005 [M] · **LEFS MDC 9 and MCID 9 points**, Binkley 1999 [M] · **CAIT ≤25 CAI threshold / 27.5 original**, Wright 2014 / Hiller 2006 [M] · **VISA-A controls exceeded 96**, Robinson 2001 [M]). **If there is no change, re-assess the diagnosis, the dose or referral** [E].

☐ **PAASS return to sport** (Smith 2021 Br J Sports Med) (Guideline): **Pain, Ankle impairments, Athlete perception, Sensorimotor control, Sport / functional performance**

☐ **Strength & Heel-Rise** [M]: **same tester, position & device** (Martins 2017 / Batista 2024) · **heel-rise men median 24 vs women 21 repetitions** (Hébert-Losier 2017)

Final FAAM / LEFS / VISA-A / CAIT _____ **Final WBLT & Heel-Rise** _____ **Final HHD Torque & Hop LSI %** _____ **Disposition** ☐ Self-Mgmt / RTS ☐ Ortho / Podiatry Refer

DISCHARGE SUMMARY · LONG-TERM SELF-MANAGEMENT · RETURN-TO-SPORT CLEARANCE

Discharging Clinician _____ **Sig.** _____